SACTCHECK · TREATMENT PASSPORT

# Nivolumab · every 2 weeks · adjuvant melanoma

## A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Nivolumab every 2 weeks. Adjuvant treatment continues for up to 12 months, subject to safety review.

These pages are reusable recording sheets; the prescribed course length is stated above. Ask for additional sheets as needed.

**Emergency: 112 / 999.** Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

**Contact oncology immediately** for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your out-of-hours pathway / nearest ED**

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00483/melanoma_adjuvant/

Full patient information, sources and visual timeline. No identifiers are encoded.

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

# My diagnosis, plan and medicines

Complete these pages with your team. Leave unknown or inapplicable items blank. This record supports the consent discussion and your written prescription.

## Diagnosis and overall plan

**Original melanoma site / pathology date**

**Stage / Breslow thickness / ulceration**

**Lymph-node or other disease sites**

**BRAF result / date / other relevant tumour tests**

**Advanced or adjuvant intent / earlier surgery or RT**

## Medicines and safety

**Anticancer medicines / actual dose / route / start date**

**Treatment received, missed or held / restart instruction**

**All other medicines / allergies / important previous reactions**

**Pregnancy-prevention or immune-alert plan / continuing precautions**

**Oncology / out-of-hours / pharmacy contacts**

NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference; original fields adapted to the Irish NCCP regimen context. No NCCN form or artwork reproduced.. Your Irish treating team confirms the diagnosis, prescription and follow-up plan.

# My checks, symptoms and support

Record what each test is checking and when the result will be discussed. Your team explains which examinations or scans are needed for your diagnosis and treatment.

| Test and purpose | Date / booking contact | Result discussion | Agreed next step |
|---|---|---|---|
| | | | |
| | | | |
| | | | |
| | | | |

Record bloods, skin/lesion examinations, scans and specialist checks separately. Tumour-marker and scan intervals are individually selected; do not infer them from the cycle calendar.

# Symptoms, skin and support

**New symptoms / onset / change / action taken**

**Lesion or wound / pain / eating / weight / daily function**

**Skin self-check plan / optional dated photograph record kept privately**

**Sun protection / practical, emotional or family support needs**

**Questions, response review and the outcomes that matter to me**

**My urgent contact plan / what my carer should do**

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NCCP 00483 V13A · ONE CYCLE, ONE PAGE

# Nivolumab · every 2 weeks · adjuvant melanoma

Cycle number: _____ Start date: _____ Next review: _____

Record the actual date, treatment received, any hold, important symptom and the next check. A review visit is not automatically a medicine dose or a completed cycle.

**Adjuvant treatment after surgery: Every 2 weeks** · copy this sheet for another cycle of the same phase.

### Nivolumab · PD-1 checkpoint immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Bowels** | ☐ Usual pattern | ☐ New change: report | ☐ Persistent diarrhoea / constipation: call | ☐ Blood, severe pain or dehydration: urgent review | ☐ Collapse or severe sudden illness: 112/999 |
| **Breathing** | ☐ Usual breathing | ☐ New cough / change: call | ☐ Worsening breathlessness: urgent advice | ☐ Breathless at rest: urgent assessment | ☐ Severe breathing difficulty: 112/999 |
| **Skin** | ☐ No change | ☐ New small change: report | ☐ Persistent rash / itch: contact team | ☐ Painful, spreading or blistering rash: urgent review | ☐ Severe skin reaction with acute illness: emergency help |
| **Fatigue** | ☐ Usual energy | ☐ Tiredness improves with rest | ☐ Rest does not help; usual tasks affected: call | ☐ Self-care difficult: urgent review | ☐ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.